جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نادية عبد الرحمن محمد عبد الرحمن
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** =
رقم البطاقة : 27609062101748 **رقم التليفون :** 01154705147 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 06-09-1976 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعه مقطعيه علي الصدر بالصبغه [10851] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نادية عبد الرحمن محمد عبد الرحمن
National ID: 27609062101748

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced chest CT

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Dm 5
سوناء السيد عبد الرحمن
سوناء سعيد محمد
أم أروى

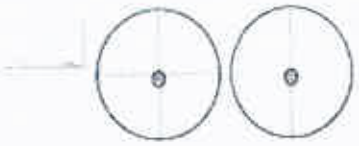
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نادية عبدالرحمن محمد عبدالرحمن Sex: Female
3- Medical number: 83154065
Old Med : 1085-2021

2-ID number: 27609062101748 Patient Age : 47 Y



Family history	Family History	Phone	Phone 01154705147 01224074943	BMI	BMI 25	REASON OF MDT	REASON OF MDT POST OPERATIVE
marital status	Marital Status	NRS Score	NRS Score 2	GROUP	GROUP A	Date of MDT	Date Of MDT 19/05/2024
Co morbidity	Co Morbidity free	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup :
ct chest: bilateral few pulmonary nodule for FU
ct abd & pelvis: free
MRI lumbo sound: L4/5 disc pritusion

BX	ER	PR	Her 2 Neu	SISH	Ki67
ILC (Bx COMMENT)	8/8 (ER COMMENT)	6/8 (PR COMMENT)	1 (Her 2 Neu COMMENT)	(SISH COMMENT)	40

T	N	M
4b	2	0

Sonomammography
rt breast localy advanced neoplastic lesion with pathological axillary & infra-clavicular LN
BIRADS V

PATIENT HISTORY
PT received NACT AC *4
Taxotere *4
RT MRM ;
free margins , LN (23/23) T2 N2

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
*Adj. AI , Zoladex for 5 yrs
PORT
Check E2 level

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: نادیه عبدالرحمن محمد عبدالرحمن **MR. :** 7853	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

A case of right breast locally advanced breast cancer (LABC) with pathological axillary and infraclavicular lymph node involvement, invasive lobular carcinoma (ILC) ER 8/8, PR 6/8, HER2 1, clinical stage CT4b N2 M0. The patient received neoadjuvant chemotherapy (NACT) with AC (doxorubicin/cyclophosphamide) followed by taxane (TXT). Subsequent radiation therapy (RT) and modified radical mastectomy (MRM) were performed in April 2024, yielding a ypT2 ypN3 (20/23) residual cancer burden (RCB) class III. Adjuvant aromatase inhibitor (AI) therapy was initiated, and the patient underwent PORT today for zoledronic acid (Zometa) only after a tooth extraction 2–3 months prior, following laboratory evaluation.

The most recent oncology note indicates that the patient is scheduled for a 6-month course of fulvestrant (FEMARA) following a free mammogram in June 2026 and a free CT chest in July 2025, with a plan to treat a few bilateral 7 mm basal pulmonary nodules with FEMARA for 3 months. No explicit cycle or week markers are documented in the notes.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

Patient Name	: الاستاذه / ناديه عبدالرحمن محمد عبدالرحمن	Request No.	: 13002810
Patient No.	: 27609062101748	Request Date	: 22/04/2024
Path No.	: 1323/24	Verification Date	: 04/05/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:
* Procedure: Right MRM measured 22x18x5 cm with covering skin ellipse measured 19x11.5 cm.
* Tumor extent: Three greyish white rubbery tumor bed sites.
* Size: 5.5 x3 x2.3 cm, 1x1 cm and 0.8x0.8 cm.
* Skin involvement: Nipple-& areola were unremarkable grossly.
* Distance from margins: 2 cm away from deep margin (least).
* Lymph nodes number and size: 23 L.Ns up to 3x3cm.
Grossing by: Dr. Sagda Elkady.

Microscopic Examination:
* Histological type: Multicentric mixed invasive lobular and duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+1) Grade II.
- Maximal tumor diameter 4.5 cm .
- Actual tumor cellularity: 15%, tumor necrosis index: 85%.
* Ductal carcinoma in situ: Present 2% (LCIS & DCIS).
* Perineural invasion: Present.
* Lymphovascular invasion: Present.
* Skin involvement: Infiltrated the base of the nipple with free epidermis.
* Lymph node involvement: Positive metastatic deposits (23/23), largest metaststic deposit was 1 cm in diameter, with extracapsular extension.
* Resection Margins: Free.
* Histologic response to therapy: Partial pathologic response.
* Microcalcifications: No.
* Additional pathologic findings: Fibroadenomatoid hyperplasia.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Rufaida /

Patient Name	: الأستاذه / ناديه عبدالرحمن محمد عبدالرحمن	Request No.	: 13002810
Patient No.	: 27609062101748	Request Date	: 22/04/2024
Path No.	: 1323/24	Verification Date	: 04/05/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis:
Right breast cancer post NACT, MRM specimen:
Multicentric mixed invasive lobular and duct carcinoma, grade II.
- Maximal tumor diameter: 4.5 cm
- Actual tumor cellularity: 15%, tumor necrosis index: 85%.
- Free resection margins
- Free skin.
- Positive lymph nodes (23/23) with extracapsular extension.
- Partial pathologic response to therapy.
- Stage: ypT2(m), ypN3.
- RCB: 3.737, RCB class III.

Signature
Prof. Amal Ahmed Hareedy.
Dr. Rufaida Abdullah.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / ناديه عبدالرحمن محمد عبدالرحمن	Request No.	: 13003406
Patient No.	: 27609062101748	Request Date	: 02/09/2023
Path No.	: 2952/23	Verification Date	: 20/09/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 31/08/2023, 4:35 pm.

Gross Examination:
- Four grayish white tissue cores up to 1.5cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive lobular carcinoma .
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: No.
* Perineural invasion: No.
* Angioinvasion: No.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive lobular carcinoma , Grade II.
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Iman Gouda
Dr. Heba Moustafa

Verified by :
Dr.Heba Hasan

Room No. (4) - First Floor Building No. 537- Nile Corniche - Sheikh Zayed - Giza, Egypt

Pathology Report
[[DOCTYPE PATHOLOGY_REPORT]]

مستشفى أورام دار السلام
قسم الباثولوجى

Pathology Report

Name: ناديه عبدالرحمن محمد عبدالرحمن Age: 46 years.

Clinical data: Right fibroadenoma.

Specimen type: Right breast mass.

Date (Received): 17/4/2021 Date(reported): 22/4/2021

Lab. Number:837 /21 Hospital Number: 1085/21

Gross Description:

A well-defined nodular mass measured 1.5x1x1cm with attached breast tissue measuring 1x1cm. Cut section revealed nodular rubbery greyish with tissue multisected and totally submitted.

Microscopic Examination:

Histopathological examination of sections prepared from the specimen received reveals a benign growth made up of proliferating small patent mammary ducts. Some of the ducts are compressed and closed, the others are patent. The ducts are lined by inner columnar and outer myoepithelial cells. The stroma in between is formed of loose fibrous connective tissue. The attached breast tissue showed diffuse stromal fibrosis and focal adenosis. There is no evidence of malignancy in sections examined.

Diagnosis:

Right breast mass. Excision:
Fibroadenoma with focal adenosis.

Prof. Dr. Lubna Omar
Dr. Abdularahman Ibrahim

Pathology Report
[illegible]

Patient Name	: الأستاذه / ناديه عبدالرحمن محمد عبدالرحمن	Request No.	: 13003406
Patient No.	: 27609062101748	Request Date	: 02/09/2023
Path No.	: 2952/23	Verification Date	: 20/09/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 31/08/2023, 4:35 pm.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (Allred score 6/8).
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).
- Ki67: 40% nuclear reaction of tumor cells.

Conclusion:
Right breast mass, Tru-cut biopsy:
Invasive lobular carcinoma, Grade II.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Score 1+
- Ki67: Positive in 40% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :
Dr.Heba Hasan

Room No. (4) - First Floor-Building No. 237- Nile Corniche - Mist El Qadema- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / ناديه عبدالرحمن محمد عبدالرحمن	Request No.	13003406
Patient No.	27609062101748	Request Date	02/09/2023
Path No.	2952/23	Verification Date	20/09/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	46 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ıo staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and ;, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, ntermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining bserved or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of ıe invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely erceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane taining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or omplete and circumferential membrane staining that is intense and within < 10% of the invasive tumor ells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor ells.

Signature
Prof. Dr. Gamal Abdelaty
Dr. Esraa Magdy

Verified by :
Dr.Heba Hasan

Room No. (6) - First Floor-Building No. 207- Nile Corniche - Nile El Qudreia- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Cairo university –faculty of medicine
Cairo university hospital
Radiology department

جامعه القاهره -كليه الطب
مستشفيات جامعة القاهرة
قسم الاشعة التشخيصية

Patient Name: ناديه عبد الرحمن محمد	Patient ID: 27609062101748
Study Date: 8/30/2023	Age: 47 Years

Bilateral Full Field Mammography with Complementary Ultrasound Examinations Revealed

clinical history: history of right breast lumpectomy.

Findings:

- *Heterogeneously dense breast (ACR c).*
- *Right breast relatively reduced size, showing diffuse architectural distortion associated with skin, trabecular thickening which corresponds by complementary US to a large hypoechoic area of altered parenchyma involving the outer quadrants and UIQ extending from 6 O'clock to 2 O'clock, associated with skin thickening, increased vascularity, peu d'orange and retracted nipple.*
- *No suspicious micro-calcifications.*
- *Complementary Ultrasound*
- heterogeneous left breast parenchyma showing scattered tiny cysts.
- *Normal left skin thickness and contour.*
- *Normal left nipple areola complex.*
- *left non-specific axillary lymphadenopathy.*
- *Multiple right axillary pathologically enlarged axillary LNs, showing eccentric hilum and diffuse cortical thickening.*
- *Multiple right infra -clavicular LNs showing effaced hilum, the largest one measures about 10 x 6mm.*
- *No pathologically enlarged right supra-clavicular LNs.*
- *No pathologically enlarged left axillary, supra or infraclavicular LNs.*

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:	
Patient Name: NDEA Abdullrahman	Date of birth: 06/09/1976
Arabic Name: نادية عبدالرحمن	Patient Gender: F
Patient RIS ID: 343769 Patient HIS ID: 123025818	Exam Date: 03/12/2023
Written by: Dr. Hoda Fathy	Approved by: Dr. Hoda Fathy

Nuclear Medicine Unit

وحدة الطب النووى

^{99m}Tc-MDP BONE SCAN

Brief history: 47 years old female patient diagnosed with managed breast cancer.

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP in the following manner:

Report:

- *No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.*
- *Spondylo-degenerative changes are seen over dorso-lumbar vertebrae.*
- *Pronounced osteoarthrosis noted over both knee joints.*
- *Both kidneys are normally visualized.*

Conclusion:

Negative study for the detection of any definite metastatic osseous deposits.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Thanks for reference

Nuclear Medicine Unit

Aya Ashraf, MSc.
Mahmoud Akram

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Abd Al Rahman Mohamed Abd Al Rahman	Date : 31/08/2023
Exam : C.T CHEST	Age : 47 Year(s)
Medical ID: 1085/2021	41568

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Findings:

- Right breast increased skin thickening associated with increased breast density.
- Multiple pathological enlarged level I and II right axillary LNs.
- Bilateral few plural based pulmonary nodules with posterior lower lobe predilection. The largest measures 7mm in the middle lobe mediastinal pleura.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest walls.
- Upper abdominal cuts are unremarkable.

OPINION:

Bilateral few pleural based pulmonary nodules, for follow up.

No evidence of pneumonia.

Dr. Ola Magdy, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]